

Controlling ‘automatic’ body functions: what the pilot evidence shows

Plain-language summary · Voluntary Body Control Research Project · 4 October 2026

Preliminary — not for citation. This summarises the project’s pilot evidence map of 3 October 2026, compiled with the help of an AI assistant (Claude). Its numbers have not yet been independently re-checked, and the project’s registered systematic review will replace it.

People can deliberately change more than 20 body functions usually thought of as automatic. But the most dramatic changes come from the fewest people and the weakest evidence.

What the pilot found

- **Most proven control works through a ‘lever’.** Ten of the 26 abilities reviewed reach the highest evidence level. Seven of the ten work through breathing, imagined movement, hypnotic suggestion, or a live readout of a body signal (biofeedback). The other three are small muscles, in the middle ear, in the eye and on the outer ear, that a minority of people can work directly, with no lever.
- **The bigger the change, the weaker the proof.** Across the 11 abilities whose effect could be put on a common scale, larger effects came with weaker evidence and with fewer people able to do them (rank correlations of -0.60 and -0.81). Part of this is how studies report: dramatic results are often one person’s best attempt, while well-proven results are group averages.
- **One exception stands out.** About 4 in 10 people can tense a tiny muscle inside the middle ear on purpose, often heard as a rumble (83 of 192 people surveyed, 43%). In 5 people tested, while the muscle was tensed a low tone (250 Hz) had to be 22 decibels louder before they could hear it. It needs no lever, it is common, and its effect is large and measured with instruments.
- **Some claims fail.** No one has been shown to stop their own heart, and there is no documented case of one person’s mind directly affecting another person’s nerves without implanted or external equipment.

Examples from the evidence map

Ability	What was shown	Who	How sure
Middle-ear muscle (‘ear rumble’)	Hearing threshold for a low tone raised 22 dB during contraction	About 4 in 10 people (43% of 192 surveyed)	High
Strength from imagined contractions	Strength gains from practice without moving (13 studies pooled)	Healthy adults, with training	High
Pain relief by hypnosis	Less pain on average; much more in highly suggestible people, almost none in the least	Most people, scaling with suggestibility	High
Slow, guided breathing	Systolic blood pressure about 4 mmHg lower, beyond the placebo effect	Adults with high blood pressure	Moderate
Breathing technique and inflammation	Lower inflammatory signals in a controlled lab challenge (IL-6 about 35% lower)	Trained volunteers; one research group	Moderate
Core temperature in g-Tummo meditation	Armpit temperature up 2.2 °C (the best of 10 people)	Expert meditators	Low
Heart-rate surge	About 300 beats per minute for about 17 seconds	One person; a book account, not peer-reviewed	Low
Stopping the heart	Not achieved; the heart kept beating	No one	Not supported

How sure, by the project’s rubric: High = measured with instruments, repeated by independent groups or pooled in a meta-analysis, with a known mechanism. Moderate = measured in controlled studies, but small samples or mostly one research group. Low = one study, a handful of people, or not peer-reviewed. Not supported = tested and not shown.

What this does not show

- **It is a pilot.** 26 of its 98 sources were read only through secondary accounts, citation records or abstracts, and no number has yet been checked by a second, independent extractor.
- **It asks whether the control exists, not whether it helps.** Health benefits, and whether anything is safe to try, are separate questions.
- **The headline pattern is exaggerated** by setting one person’s best attempt beside group averages; its direction is more trustworthy than its exact size.

What happens next

A registered systematic review will re-extract every ability under rules fixed in advance, with two independent extractors, human sign-off and every number traced to its source. Its results, not this pilot, are what the project will report.

Source: Voluntary Body Control Research Project, pilot evidence map (export of 3 October 2026, docs/research-project.md in the private project repository). Program lead: Sean Borycki. Prepared 4 October 2026.